May I Strategy.

Customer Segmentation

May I, LLC will focus on the Healthcare vertical because its founders have domain knowledge and expertise in building technology for healthcare. Chami Rupasinghe and Teng Li have both worked at Microsoft, starting in the prestigious Microsoft Research division building healthcare platform technologies and then in Microsoft Azure Health and Life Sciences building its Ambient AI technologies for providers. **Chami and Teng have unique experiences building scalable, enterprise grade, healthcare AI technologies**.

A focus in **healthcare offers a natural compliance moat** that deters companies that do not have the experience in the space or that build general purpose technologies to avoid the segment. The healthcare industry is governed by a complex and stringent set of compliance and regulatory requirements, most notably the **Health Insurance Portability and Accountability Act (HIPAA)**, which mandates strict standards for the handling, storage, and transmission of electronic Protected Health Information (ePHI); these **requirements make it extremely challenging for generic technology platforms to gain relevance or penetrate the healthcare space**, as such platforms often lack essential features such as robust encryption, detailed audit trails, and granular access controls needed for HIPAA and privacy compliance.

Customer Profile

May I's **primary customers will come from the Retail Healthcare sector**. This group includes businesses/practices such as **Medical Aesthetics (plastic surgery, Med Spas, dermatology clinics, weight loss clinics, etc..), Ophthalmology, Pain Management, Podiatry, IVF clinics, dental and orthodontic practices, mental health groups, and veterinary clinics**. Organizations in this segment face intense competition and a high degree of patient choice, making each patient's lifetime value considerable. Their services often involve high-intent, elective procedures and typically follow a private-pay or hybrid payment model. Therefore, May I will NOT be supporting Urgent Care centers, Emergency Rooms, Dialysis centers, Primary care practices, Medicare-focused or Medicaid clinics, General Radiology or Clinical Pathology Labs.

The transaction value of each procedure is high[1] therefore businesses in this segment have more disposable income to spend. As these businesses grow, they're focused on addressing the "leaky bucket". They want to capture more leads with top of the funnel marketing activity, they don't want to lose leads (leaky bucket), and they want to increase their conversion rate.

Customer Challenges / Friction

Our core strategy is to position the platform as a **Patient Conversion and Revenue Integrity Engine** for high-intent, elective healthcare verticals. We aim to **capture the "Invisible Loss", the significant revenue lost to missed calls, slow response times, and administrative friction** by merging UCaaS, CRM, and conversational AI into a single, automated workflow. By prioritizing "Speed-to-Lead," **we ensure that every patient inquiry is engaged within seconds**, effectively removing the front-desk bottleneck and preventing lead leakage to competitors.

To achieve market dominance and high stickiness, the product suite will evolve from a communication tool into a clinical operating system. This involves transitioning beyond simple message capture to intelligent triage and automated follow-up, ultimately closing the loop via bi-directional EHR integration and text-based payments. Our strategic value proposition **shifts the narrative from "operational efficiency" to "direct revenue generation," making the platform an indispensable financial asset rather than a cost center.**

Crucially, we **leverage AI to deliver a personalized, white-glove experience** that enables practices to differentiate themselves in a crowded market. By utilizing historical CRM data (communication history and sentiment) and EMR data (patient appointment, and encounters), the AI acts as a digital concierge—providing a bespoke, high-touch interaction that anticipates patient needs and maintains brand premium from the first point of contact. This transforms the patient journey from a cold administrative process into a tailored experience, moving the platform beyond a "utility" and into a strategic brand asset that drives both patient loyalty and direct revenue growth.

Product Stack

Agentic AI platforms that enable communications and reasoning are the newer of all technologies and its capabilities will be quickly commoditized. Agentic AI by itself cannot be relied upon as a differentiator. Existing CRMs, EMRs, and UCaaS tools will infuse AI into their technology stack. A “plug-in strategy” will have a diminishing market as customers will opt for embedded native products and the tools themselves will have gates to limit the functionality of integrated products. **To be an indispensable tool for a business, it must be an integral part of the end user lifecycle. Therefore, it must host ground-truth data, and it must integrate with proprietary systems (EMR), where the transactions occur**. In healthcare the EMR is that critical system, and for now, we will assume this is out of scope for development.

The next critical system for any business is a communications system. A phone line, internet, communications hardware (telephones) and a way to set rules (office hours, routing, etc..). These capabilities are packaged as a universal communications as a service (UCaaS) system and typically have a phone tree (IVR) system that customers loathe (Press 1 for …). It can be expected that UCaaS systems will incorporate some GenAI capabilities.

The next most important system is the CRM system where patient communication is documented. Retail healthcare practices often purchase a separate CRM system because EMRs are not designed to drive marketing. The significant manpower that is required to qualify leads, intake patients, schedule appointments, and collect payment can be automated via an agentic platform. **Owning customer conversation history means owning customer intent**, which is critical for targeting and cultivating a relationship with customers to monetize that long term value (LTV) through marketing.

May I, LLC will prevent lost revenue and increase the conversion rate for healthcare customers by offering a foundationally agentic AI feature set that encompasses AI comms, CRM, and UCaaS.

Technology

Conversational AI – A conversational agentic solution manages incoming customer calls, provides information regarding the business, service providers, or products, routes calls appropriately, conducts customer intake, and schedules appointments. This solution enables the organization to increase operational capacity efficiently while minimizing additional overhead.

CRM – For the company, an agentic CRM engine becomes the anchor product because it will be the source of truth for customer conversations that is indispensable to the target customer segment. The CRM will document new customer intent that the AI or front-office staff captures, it enables AI multi-modal outreach and lead qualification, and most importantly personalization. As a system that hosts all customer communications, it becomes a business-critical system versus an add on. Creating success for the customer will require CRM integration with PMS/EMR to close the loop on the transaction.

Unified Communications-as-a-Service (UCaaS) - May I's Unified Communications-as-a-Service is designed to provide healthcare practices with a robust, enterprise-grade communication platform that integrates seamlessly with CRM and AI-driven workflows. The system will offer essential features such as the ability to purchase dedicated business phone numbers, customize business hours for tailored responses and intelligent call routing, advanced call routing capabilities to ensure inquiries reach the right provider or department, voicemail and message capture with automatic transcription to text, user-specific inboxes for secure and organized communication. By combining these features, May I's UCaaS solution eliminates traditional front-desk bottlenecks, guarantees that no patient inquiry is missed, and supports rapid, reliable engagement with both patients and staff.

Technical Moat

A key differentiator of May I will be its **AI Evaluation and Performance Monitoring Engine**, which continuously measures, tests, and optimizes every AI-driven call flow against the customer's desired outcomes. Rather than treating conversational AI as a static workflow, May I instrumentally tracks where callers disengage, hesitate, or drop off within each step of the conversation. The platform enables controlled A/B testing of prompts, routing logic, and messaging sequences, allowing

practices to systematically identify what drives higher conversion, appointment completion, and revenue capture. By closing the loop between conversational design and real-world outcomes, May I transforms AI call flows into a continuously improving revenue system, one that adapts over time and compounds performance gains in a way that SDK based generic AI agents cannot.

May I, LLC will build technologies that enable a differentiated and elevated experience of the solutions listed below. One such example is **voice as biometric identification that can be used as one (of multi) factor of authentication** to verify the caller before sharing PHI. Security requirements will be thoughtfully balanced with customer experience, as May I adopts a prescriptive approach, implementing lighter authentication for routine actions like appointment changes, while enforcing stricter security protocols when handling sensitive medical information.

We will implement an **MCP (Model Context Protocol) Architecture** to transform our CRM from a static database into a **Dynamic Agentic Environment.** By placing an MCP server in front of our proprietary data, we decouple the AI's 'reasoning' from the 'integration logic.' This allows us to rapidly deploy highly personalized, white-glove AI agents that can autonomously navigate clinical histories, verify insurance, and manage payments with 100% factual accuracy. This architecture ensures our platform is 'future-proof,' allowing us to swap underlying LLMs or integrate new EHRs without ever rebuilding our core patient interaction workflows.

The **Patient Intent Graph (PIG)** will be May I's proprietary intelligence layer that continuously models and predicts patient intent across every interaction in the care journey. This is made possible by May I's companion CRM, which captures and unifies patient interactions across all communication modalities. By owning the full conversational history across voice, SMS, chat, web, and follow-up outreach, PIG analyzes signals such as language, sentiment, urgency, timing, and historical outcomes to infer a patient's readiness to convert, likelihood to proceed, and need for human escalation, often before that intent is explicitly stated. This dynamic graph enables May I to prioritize high-value inquiries, personalize engagement in real time, and allocate staff attention where it has the greatest revenue impact. Over time, PIG compounds into a defensible data asset, linking intent to appointments, procedures, and payments, and transforming fragmented conversations into actionable revenue intelligence that cannot be replicated by point solutions or standalone AI agents.

Solutions

24/7 Personalized Concierge – May I's 24/7 Personalized Concierge offers healthcare practices a transformative advantage by providing an always-on, AI-powered agent that guarantees no patient inquiry or potential lead is ever missed, regardless of the time of day or staff availability. In high-intent, elective healthcare sectors where competition is fierce and patient choice is paramount, the ability to instantly engage every inquiry is critical for maximizing conversion rates and protecting revenue. By merging unified communications, CRM, and conversational AI into a single automated workflow, the concierge eliminates the traditional front-desk bottleneck, responds to patient questions within seconds, and routes calls or messages to the appropriate provider or department. This not only prevents lead leakage to competitors but also ensures a consistently high standard of service, reinforcing the practice's reputation for responsiveness and reliability. As a result, the 24/7 Concierge becomes an indispensable asset for practices aiming to capture every opportunity, drive direct revenue growth, and maintain a premium brand

presence in a crowded market. The agentic concierge can be accessed via phone call, text message, or a chat application on the home page of the customer's website.

Personalization – This is essential for businesses providing luxury services. Personalization requires having a history of the customer transactions and communications to identify intent. To get this transaction history, integration with the proprietary system is required. However, the proprietary system with the transaction history does not have the patient's conversation history, which is why a CRM is important. Conversation history will be multi-modal including, phone calls, text messaging, chats, social media DMs.

Front Office Co-Pilot - Augments—not replaces—the human front desk by transforming every staff interaction into an informed, high-conversion engagement. When a call or message is handed off from AI to staff, the Co-Pilot presents a real-time intelligence layer via a streamlined Web UI. As the conversation unfolds, the AI live-transcribes the dialogue, instantly identifying and capturing key data points such as symptoms, procedure interest, and intent.

Moving beyond mere transcription, the Co-Pilot acts as a functional bridge to the EMR/EHR. Through a front-end integration, a specialized AI agent "reads" the captured values and automatically populates patient profiles, insurance details, and intake fields directly into the web interface of the clinical software. This eliminates the cognitive load of manual data entry and the fragmented inefficiency of toggling between systems.

Staff are no longer tethered to a keyboard to "catch up" on documentation; instead, they are empowered with prior transaction history, appointment context, and sentiment analysis at a glance. By automating the "clerical" side of the call in real-time, the Co-Pilot enables the front office to lead with confidence, empathy, and relevance. In high-value, elective healthcare settings, this ensures every interaction feels personalized and premium—maximizing trust while accelerating revenue capture.

- Zero API Friction: By integrating via the web UI (front-end), you don't have to wait for EMR companies to grant expensive API access. The AI interacts with the EMR just like a human would, but at machine speed.
- Reduced "Wrap-Up" Time: Usually, a receptionist spends 2–3 minutes *after* a call typing notes. With this, the work is done the moment the "Goodbye" is said.
- Improved Data Integrity: AI doesn't make typos. It pulls the insurance ID and patient name directly from the transcript, ensuring the EMR remains a "single source of truth" without human error.

Patient Intake and scheduling – The AI-driven intake solution can capture and digitize patient information from a variety of sources, such as phone calls or uploaded ID cards (drivers and insurance) ensuring that data is accurately extracted and immediately integrated into the practice management systems. This is usually a tedious task that the front office performs and it is quite tedious for the patient as well; Handing it off to an AI saves the business time and minimizes manual data entry errors. By leveraging AI and Optical Character Recognition (OCR), May I transforms this process into a seamless, automated experience that reduces administrative friction for the front office and the customer and accelerates speed-to-lead.

Appointment Management - May I's Appointment Management solution transforms scheduling from a manual, error-prone task into an intelligent, automated revenue engine. By integrating AI-driven scheduling with real-time availability and CRM context, the platform ensures appointments are booked quickly, accurately, and with minimal friction—whether initiated by AI or staff. This reduces abandoned inquiries, prevents double-booking, and optimizes provider utilization, directly improving revenue throughput. More importantly, appointment data becomes a foundational signal for downstream personalization, follow-ups, and lifecycle marketing, ensuring scheduling is not an isolated administrative task but a core component of the patient conversion journey.

Appointment Reminders - Automated Appointment Reminders reduce costly no-shows while reinforcing the practice's premium, concierge-level experience. Leveraging multi-modal outreach (text, voice, and email) and informed by CRM and appointment context from the EMR, reminders are delivered at the optimal cadence and tone for each patient. Beyond basic confirmations, reminders can include preparation instructions and payment expectations—lowering friction while preserving staff time. The result is higher appointment adherence, improved calendar efficiency, and predictable revenue realization without increasing operational overhead.

Personalized Care Journey (Pre-Op & Post-Op Outreach) - Delivers continuous, high-touch engagement before and after procedures, ensuring patients feel informed, supported, and confident at every stage of their care. By leveraging historical communications, appointment context, and procedure-specific workflows, May I automates tailored outreach that prepares patients ahead of their visit, delivering instructions, answering common questions, and proactively addressing anxiety while continuing post-procedure with recovery check-ins, care guidance, and easy escalation paths. This end-to-end engagement reduces last-minute cancellations, no-shows, and operational disruption, while lowering inbound support volume and improving clinical outcomes. Strategically, the Personalized Care Journey transforms episodic procedures into long-term relationships, reinforcing a white-glove brand experience, increasing patient retention and referrals, and protecting revenue across the full patient lifecycle.

Insurance Verification - Automates one of the most time-consuming and error-prone front-office workflows by transforming manual portal navigation into a seamless background process. May I validates coverage and handles automated pre-authorizations early in the patient journey. Our platform employs a dual-layer automation engine: we read and write data via secure APIs for high-reliability "read/writes" while simultaneously deploying AI Agents to navigate complex payer portals via the Front-End UI.

Specifically for insurance pre-authorization, this hybrid approach enables the practice to scale across hundreds of different insurance carriers without waiting for custom API development. The AI analyzes the unique requirements of each insurance site, identifying necessary fields and auto-populating clinical data, CPT codes, and patient history. To protect against "broken automations," the system employs UI-Hashing technology, creating a cryptographic fingerprint of the portal's interface. This allows the AI to instantly detect if a payer has updated their web UI, triggering a proactive adjustment before the workflow is interrupted. By shifting the burden of portal data entry from humans to AI, the front office can pivot from being "data processors" to "patient advocates"—accelerating cash flow and protecting margins by eliminating avoidable friction at the point of care.

Payment Collection - May I's Payment Collection capabilities close the loop between patient engagement and revenue realization. By integrating payment workflows directly into AI-driven communications and appointment lifecycles, the platform enables practices to request, collect, and reconcile payments seamlessly via text or digital links. This reduces delays, improves collection rates, and minimizes awkward, staff-mediated financial conversations. Payments are no longer a back-office afterthought—they become a natural, frictionless extension of the patient experience that directly supports revenue integrity.

Lead Qualification - Ensures that staff time is focused on high-intent, high-value opportunities. Using conversational AI, intent scoring, and historical CRM signals, May I automatically qualifies inbound inquiries, identifies readiness to convert, and routes leads appropriately. Low-intent inquiries are nurtured through automated follow-ups, while high-intent leads are prioritized for immediate human engagement. This protects the practice from lead leakage, increases conversion rates, and ensures that every inquiry is handled with the right level of urgency and personalization.

Membership Portal and Management - Enables practices to create predictable, recurring revenue streams while deepening patient loyalty. May I provides a centralized system for managing memberships, subscriptions, benefits, and communications—fully integrated with CRM and billing workflows. Patients gain transparency and convenience, while practices gain operational efficiency and improved lifetime value per customer. Memberships shift the relationship from transactional to ongoing, aligning perfectly with the platform's long-term revenue integrity strategy.

Social Media Integration for Top-of-Funnel Marketing - Allows practices to capture and convert high-intent leads directly from top-of-funnel channels. May I connects inbound social inquiries to AI-driven engagement workflows, ensuring rapid response times and immediate qualification. Rather than routing leads into disconnected inboxes, every interaction is captured in the CRM, enriched with intent data, and nurtured toward conversion. This closes the gap between marketing spend and measurable revenue outcomes.

Database Marketing - Turns historical patient data into a strategic growth asset. By leveraging conversation history, appointment data, and engagement signals, May I enables targeted, personalized outreach campaigns that drive repeat visits, upsells, and reactivation. Unlike generic blast marketing, database marketing is context-aware and intent-driven, ensuring messages feel relevant, timely, and premium. This increases patient lifetime value while preserving brand trust.

Funnel Analytics - Provides end-to-end visibility into where revenue is created—or lost—across the patient journey. May I tracks every interaction from first contact through conversion, identifying drop-off points, response-time gaps, and performance bottlenecks. These insights allow practices to quantify "Invisible Loss," optimize workflows, and make data-driven decisions that directly impact revenue. Analytics transform intuition into accountability, positioning the platform as a financial intelligence system rather than an operational tool.

Storefront Integration for Product Sales - Extends revenue beyond clinical services by enabling seamless product sales tied to patient engagement. May I integrates product catalogs, recommendations, and payment workflows directly into post-visit and ongoing communications. Whether skincare,

supplements, or recovery products, recommendations are contextual and personalized—driving incremental revenue without additional staff effort. This transforms the platform into a holistic monetization engine that captures value across the entire patient lifecycle.

Go to Market Strategy

Channel: Sales team

May I will pursue a direct, high-touch sales motion focused on engaging owners and operators of retail healthcare practices, where purchasing decisions are centralized and tightly coupled to revenue outcomes. The initial sales motion will be founder-led to refine messaging, validate ICP assumptions, and establish repeatable proof points tied to recovered revenue and conversion lift. As traction scales, May I will build a specialized sales team composed of account executives with experience selling into elective healthcare and practice-owned businesses. The sales narrative will be anchored in loss aversion and measurable ROI—demonstrating how missed calls, slow response times, and front-desk bottlenecks directly translate into lost procedures and unrealized revenue. This approach positions May I not as a software expense, but as a revenue-producing asset that pays for itself through recaptured demand and increased conversion.

Channel: Direct (Top of the funnel marketing)

May I will use its own platform as the primary engine for top-of-funnel marketing, creating a self-reinforcing feedback loop that showcases the product's capabilities while generating qualified demand. Inbound marketing efforts—across website chat, SMS, voice, and social channels—will be fully instrumented through May I's AI communications and CRM stack to ensure immediate engagement, automated qualification, and rapid follow-up. Every inbound inquiry will be treated as a revenue event, with intent scoring, response-time measurement, and conversion tracking applied from first touch through close. By dogfooding the platform, May I not only reduces customer acquisition costs but also continuously refines its messaging, qualification models, and conversion workflows using real performance data, strengthening both marketing efficiency and product differentiation.

Channel: Partnerships

May I's **partnership strategy will focus primarily on healthcare marketing agencies** that specialize in top-of-the-funnel demand generation for retail healthcare practices. These partners are directly responsible for driving inbound patient inquiries and are uniquely positioned to benefit from May I's ability to engage, qualify, and convert leads in real time. By embedding May I into their clients' marketing stack, agencies gain a differentiated way to prove ROI beyond clicks and impressions—demonstrating how demand translates into booked appointments and realized revenue—while May I benefits from a scalable, low-CAC distribution channel that embeds the platform at the earliest and most valuable point in the patient journey. Secondarily, May I will pursue selective partnerships with EMR and Practice Management System vendors

where deep, bi-directional integration enhances workflow continuity and revenue capture, reinforcing May I's role as the system of engagement while the EMR remains the system of clinical record.

Messaging

Commercial Positioning: "The Revenue Integrity Strategy" - Our primary go-to-market strategy centers on Loss Aversion as the initial hook, followed by Revenue Generation as the long-term value driver. We will position the platform not as a cost-cutting utility, but as a Revenue Integrity Engine that captures "The Invisible Loss" currently leaking from the practice's bottom line.

1. The Strategic Hook: Capturing "The Invisible Loss"

The entry point for all market engagement will focus on Loss Aversion. We will demonstrate that the practice is actively losing significant revenue due to the "Speed-to-Lead" gap—the high-value surgical inquiries that go to competitors when the front desk is busy.

Narrative: We move the conversation from "saving money" to "recapturing lost investment." By highlighting that every missed call or delayed text is a $5,000+ loss, we create immediate psychological urgency and a "need-to-act-now" mindset that "cost savings" alone cannot trigger.

2. The Value Pivot: From Recovery to Growth

Once the "leak" is identified, the strategy pivots to Revenue Generation through our AI-driven "White-Glove" experience.

Narrative: The platform is positioned as a growth lever that increases the Lifetime Value (LTV) of every patient. By providing a personalized, concierge-level experience via AI, the practice differentiates itself from "commodity" clinics. We are not just capturing leads; we are nurturing them into high-margin surgical procedures through automated, high-touch engagement that the front desk is physically unable to provide.

3. The ROI Floor: Efficiency as the Justification

We utilize Operational Efficiency (cost savings) as the "safety net" to remove financial risk from the purchasing decision.

Narrative: We frame the reduction in staff "phone tag" and administrative double-entry as the ROI Floor. This ensures the platform "pays for itself" through labor arbitrage alone. By automating 40% of the front-office workload, the technology becomes a self-funding asset, allowing the revenue gains from increased conversions to be viewed as pure, incremental profit.

Appendix

Note 1 US Retail Healthcare Market Analysis (2025 Data)

Category	Est. No. of Practices	Avg. Providers	Avg. Transaction Value	Avg. Annual Transactions / Practice	Avg. Annual Revenue / Practice	Total Addressable Market
Ophthalmology	3,700	3–6	$ 3,500	$ 5,500	$ 19,250,000	$ 71,225,000,000
Plastic Surgery	4,500	1–3	$ 8,000	$ 1,400	$ 11,200,000	$ 50,400,000,000
Orthodontics	16,000	1–2	$ 6,200	$ 325	$ 2,015,000	$ 32,240,000,000
Medical Spas	10,500	2–4	$ 536	$ 2,940	$ 1,575,840	$ 16,546,320,000
Psychiatrists	11,000	5–15	$ 260	$ 3,800	$ 988,000	$ 10,868,000,000
Podiatry	9,886	1–3	$ 185	$ 5,000	$ 925,000	$ 9,144,550,000
Fertility (IVF)	500	3–5	$ 16,500	$ 950	$ 15,675,000	$ 7,837,500,000
Weight Loss	4,000	1–2	$ 350	$ 2,500	$ 875,000	$ 3,500,000,000
Pain Mgmt	900	2–5	$ 320	$ 5,000	$ 1,600,000	$ 1,440,000,000

Category	Practice Count Derivation	ATV Derivation	Volume Derivation
Plastic Surgery	Based on **7,752** active board-certified diplomates (ABPS) clustered into ~4,500 private surgical groups.	Calculated as a weighted average of high-volume surgeries (Breast/Lipo at $5k–$10k) and non-surgical procedures ($1,100).	Derived from Aesthetic Society reports of ~1,400 surgical cases per average multi-surgeon practice.
Fertility (IVF)	Verified via **SART** (Society for Assisted Reproductive Technology) registries, which track ~500 clinic locations.	Represents the 2026 average cost of one IVF cycle including medication and lab fees.	Based on national data of ~400,000 annual cycles distributed across 500 clinics.

Orthodontics	Calculated from AAO data and DSO affiliation trends, forecasting 16,000 targetable specialty offices by 2026.	Standard industry "Case Start" value for traditional braces or clear aligners.	Derived from the average of 350 "new starts" per provider per year (excluding routine check-ups).
Ophthalmology	Based on AAO and CMS billing registries for standalone specialty surgical centers.	Blended rate for premium IOL cataracts ($3,500+) and routine diagnostic exams.	Based on MGMA productivity benchmarks for high-throughput surgical specialists.
Medical Spas	Projected from **AmSpa** 2024 data (10,488) with a 13% CAGR growth forecast into 2026.	Reflects the average patient ticket for a "Botox + Filler" or "Energy Device" visit.	Total annual revenue divided by ATV, supported by AmSpa "State of Industry" throughput reports.
Psychiatry (MD/DO)	Based on the 11,000 outpatient clinical entities distinct from solo-therapist offices.	Blended rate for Initial Evals ($350+) and Med-Check follow-ups ($175) using 2026 Medicare CF updates.	Based on a "Clinical Full-Time" load of 15 patients per day (3,100+ annually).
Pain Management	Derived from Definitive Healthcare's tracking of interventional pain group practices.	Weighted average of follow-up visits and high-value interventional procedures (Epidurals/SCS).	Based on practice management data for multi-provider pain clinics.

Podiatry	Based on 2025 IBISWorld business census data for Podiatric Medicine firms.	Standard CMS/Private Pay reimbursement for common office-based procedures.	High-volume clinical model (4,000+ visits per year) based on APMA practice surveys.

Source Reference Links

1. **Plastic Surgery:** ABPS 2025 Active Diplomate Statistics | Aesthetic Society 2024 Procedure Report
2. **Medical Spas:** AmSpa 2024 State of the Industry Executive Summary
3. **Psychiatry:** AMA 2026 Physician Payment Schedule & Conversion Factors
4. **Fertility:** Fertility Bridge: 2025 Industry Review
5. **Orthodontics:** Fortune Business Insights: Orthodontics Market 2026-2034
6. **Ophthalmology:** Review of Ophthalmology: 2026 Coding & Reimbursement Update
7. **Podiatry:** IBISWorld: Podiatrists in the US Business Statistics
8. **General Healthcare Outlook:** Deloitte 2026 US Health Care Outlook

Creating Moats

1. Closed-Loop Revenue Attribution Engine (CRAE)

What it is: A system that deterministically attributes *every dollar of collected revenue* back to:

- first contact channel (call, SMS, website, social),
- response latency (speed-to-lead),
- agent behavior (AI vs human),
- messaging content and sequence.

Why it's defensible

- Requires deep integration across UCaaS + CRM + payments + EMR/PMS
- CRMs can track leads; payment systems can track money — very few can provably connect the two
- Attribution data compounds over time and becomes irreplaceable

How it becomes a moat

- Your models learn *which sequences convert $5k vs $15k procedures*
- Practices can't churn without losing historical revenue intelligence
- Enables pricing tied to incremental revenue created, not seats

Implementation hook

- Event-sourced ledger of:
    - inbound interaction
    - qualification decision
    - appointment booking
    - procedure completion
    - payment settlement

2. Procedure-Aware Conversational Models — *Healthcare-Specific AI Moat*

What it is - Fine-tuned or instruction-layered conversational agents that understand:

- procedure-specific risks, prep, and post-op flows
- provider-specific preferences
- payer / private-pay nuances

Why it's defensible

- General LLMs lack procedure-level operational intelligence
- EMRs store clinical facts, not conversational pathways
- Requires long-term real-world interaction data

How it compounds

- Each practice improves the system's understanding of:
    - patient objections
    - anxiety patterns
    - drop-off reasons
- Cross-practice anonymized learning (HIPAA-safe) strengthens models

Why Twilio / CRMs won't build this

- Too vertical
- Too risky
- Too dependent on healthcare nuance

3. Patient Intent Graph (PIG) — *Data Moat*

What it is - A continuously evolving graph that models:

- patient intent signals (questions, sentiment, urgency)
- timing (how long between touches)
- outcome (converted, lost, delayed)

Why it's defensible

- Intent is inferred, not explicitly stated
- Requires owning raw conversation data, not summaries
- Improves only with longitudinal exposure

Strategic value

- Predicts *who will convert before staff knows*
- Drives:
    - lead prioritization
    - escalation timing
    - personalized pricing / financing offers

Moat trigger

> "If we leave May I, we lose our institutional understanding of patient intent."

4. Dynamic Trust & Friction Engine (DTFE) — *Security + CX Moat*

What it is - A real-time system that adjusts:

- authentication strength
- identity verification
- payment friction

…based on risk + intent + context.

Examples

- Returning patient + appointment reschedule → no authentication
- New caller + pricing + insurance → voice biometric + OTP
- Payment collection → step-up verification

Why it's defensible

- Balances HIPAA security with conversion optimization
- Requires comms, CRM, AI, and payments in one loop
- Most systems are binary (secure vs not)

Result
Higher trust and higher conversion — rarely achieved together.

5. AI-Driven Front Office Load Balancer — *Operational Systems Moat*

What it is - A real-time optimization engine that decides:

- which conversations go to AI
- which escalate to humans

- when to interrupt staff
- how many concurrent conversations are safe

Why it's defensible

- Requires:
    - call telemetry
    - staff behavior data
    - conversion outcomes
- Learns *how each practice actually operates*

Outcome

- 40–60% workload reduction
- No degradation in patient experience
- Staff performance improves over time

This is far beyond IVR or call routing.

6. Revenue-Weighted AI Decisioning — *Pricing & Control Moat*

What it is - Every AI action is optimized not for correctness—but for **expected revenue impact**.

Examples

- $15k surgery inquiry → immediate human escalation
- $200 follow-up → AI completion
- High LTV returning patient → concierge-level flow

Why it's hard to copy

- Requires historical revenue + outcome data
- Requires confidence in AI decisioning
- Most platforms optimize for efficiency, not revenue

Strategic payoff
This enables:

- premium pricing
- executive-level buying decisions
- board-level ROI conversations

7. Practice-Specific Operating System Memory — *Stickiness Moat*

What it is - A long-lived, evolving "memory" of:

- how *this* practice converts patients

- language that works
- offers that close
- staff behaviors that succeed

Why it's a moat

- Not portable
- Not exportable
- Not replicable elsewhere

It becomes:

> "This is how *our* practice works.